When Group Gets Stuck

39 facilitator cards, Safety first, and the handoff page

From When Group Gets Stuck: A Practical Response Guide for Adult IOP and PHP Skills Groups, by Heba El Garhy (Thoughtkind Press, 2026). For staff of the purchasing program. Read the full case in the book before relying on its card. These cards support your clinical judgment and your program's procedures; they don't replace them.

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Safety first

Turn here before any card when safety might be in question. This page doesn't replace your program's crisis, emergency, or medical procedures. It helps you notice when it's time to use them. Scenarios 33 to 36 walk through four of these situations in detail.

Stop the lesson when you notice

• Words that suggest danger: "I don't know whether I can get through tonight," talk of wanting to die or not being safe, or a threat toward someone else.

• A change in alertness: new confusion, trouble staying awake or being woken, slurred speech, unsteadiness, or signs of possible intoxication or overdose.

• A possible physical emergency: chest pain, trouble breathing, unusually slow breathing, fainting, or a seizure.

• Someone saying they are unsafe right now or have nowhere safe to go tonight. Get them help today. A housing problem on its own isn't a medical emergency, but it can't wait for the end of the week either.

Then, every time

• Stay with the group while you get help. Never send a member to fetch, escort, find, watch, or interpret for someone. If the room itself is unsafe, follow your emergency or evacuation plan instead of keeping everyone seated.

• Call for help now, the way your program does it. If nobody answers, use your backup. Help has arrived when a named person tells you they've taken it on and what happens next. A message you sent is not a handoff.

• Keep it private. Ask only what you need to know right now to keep someone safe. The fuller conversation happens privately, with someone else staying with the group. Don't ask for a history or details in front of others.

• Don't finish the lesson first, and don't let the end of the session end the response. Ask the person to stay with you until staff arrive, unless your procedure says otherwise.

• For a possible medical emergency, start your emergency response immediately. Don't wait for a routine callback.

• On video, use the location and phone number you confirmed at the start of the session. A camera left on isn't the same as someone paying attention.

• If anyone is in immediate physical danger, use your emergency procedure at once. Your job is to get help, not to restrain anyone.

In the United States, the 988 Suicide & Crisis Lifeline (call or text 988) is open 24/7. It adds to your program's response; it doesn't replace it or emergency medical care.

When you call for help, say

Who you are and where you are; what you saw and what was said; where the person is now; what you need; and that you are staying with the group.

My program's routes

Help during group: _____

Backup if no answer: _____

Emergency procedure: _____

Medication questions: _____

Language services: _____

Standalone quick reference

When one member needs more time

Scenario 1 · Adult IOP · Planned skill: DBT STOP

Notice

Ten minutes into check-in, Denise has talked fast for four minutes about a family fight and a housing problem. Others are waiting. She hasn't mentioned harm, but keep listening.

First response

"Denise, that sounds like a hard night. I'm going to pause the details so everyone gets a turn. Would you like to talk with staff after this, without explaining here, or just listen for now?"

Choose the next step

• Engages: Offer private practice, the made-up example, or watching.

• Objects or mentions housing: Acknowledge it. Ask one yes-or-no question about tonight. Get practical help or a clinical assessment, whichever fits. An urgent need doesn't wait for the lesson.

• Peers offer advice: "Let's hold off on advice. It protects Denise's privacy and everyone's check-in time."

• Goes quiet: Offer listening and a check-in you can deliver. Quiet isn't the same as better.

• Possible harm or medical concern: Stop redirecting. Use your assessment or emergency procedure.

Return to STOP

First make sure Denise knows when she'll get help; housing doesn't have to be solved in the room. Then use a made-up angry-text example: Stop, Take a step back, Observe, Proceed mindfully. Pause before sending, notice the urge, choose a step that fits the goal. Stepping back doesn't mean leaving. Nobody has to practice to get help.

Support and record

With a co-facilitator, one of you steps out and one stays. Alone, stay with the group and call for staff. Never leave the group on its own. Promise only follow-up that exists. Record what support happened and whether she practiced or watched.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

Nobody answers the opening question

Scenario 2 · Adult IOP · Planned skill: DBT Describe

Notice

Nobody answers a broad question about last session. Two are new. They may be thinking, confused, facing an access barrier, or choosing not to speak. You don't know which.

First response

"I asked too much at once. Let me show one example. No personal story needed, and I won't call around the room."

Choose the next step

- Someone responds: Name the detail they offered. Don't ask them to explain everyone's silence.
- Still quiet: Contrast A, "The cup is beside the notebook," with B, "Someone left it there because they don't care." People can say A or B, signal, or decide privately. Nobody has to answer or write.
- Access barrier: Change the example or format and check that it helps. Don't assume everyone can see the cup.
- Interpersonal concern: Address reported harm before using practice to move on.
- Changed alertness or responsiveness: Use your assessment or emergency procedure. Ordinary quiet is different.

Return to Describe

Model putting words to a detail without adding motives. Invite another description, private practice, or watching. A thought can be labeled as a thought without arguing with it. Don't keep repeating the question to squeeze out an answer.

Support and record

With a co-facilitator, one steps out, one stays. Alone, stay with the group and call for staff; never leave it on its own. Offer only follow-up you can deliver. Record what you observed. Private practice and its effects can't be confirmed.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

A disagreement becomes personal

Scenario 3 · Adult IOP · Planned skill: DBT GIVE

Notice

Elena labels Marcus during a disagreement. Marcus objects; they talk over each other. Stop the attack. The insult and the reply aren't equal.

First response

"Elena, I'm stopping the personal comment. You can disagree with the suggestion without labeling Marcus. Marcus, you don't need to defend yourself or explain more."

Choose the next step

- Label withdrawn: Marcus can speak on the topic, listen, or ask for support. Don't declare it repaired.
- Defends it: Offer listening, or disagreement about the strategy without a personal label.
- Repeats the attack: Pause the discussion and call for staff. Don't debate the label.
- Peers take sides: "Hold on. I'm addressing this. We won't ask the group to judge either person."
- Marcus goes quiet: Offer support you can deliver, without asking him to reassure or mediate.
- Threat or significant clinical change: Use your assessment or emergency procedure.

Return to GIVE

Resume only when attacks have stopped and support needs are handled. GIVE doesn't mean putting up with mistreatment or staying in an unsafe relationship. Introduce Gentle, Interested, Validate, and Easy manner with colleagues who disagree about an agenda. Model acknowledgment and curiosity. Don't replay the conflict.

Support and record

With a co-facilitator, one steps out, one stays. Alone, stay with the group and call for staff; never leave it on its own. If hostility continues, stop debating, update responding staff, and follow your escalation plan. Don't delay assessment. Record conduct and support in each member's own record, without naming peers.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

A member declines the activity

Scenario 4 · Adult IOP · Planned skill: DBT Check the Facts

Notice

Tom declines a personal worksheet, worried about disclosure and being told he's wrong. Refusing isn't grounds for removal, and he can still learn.

First response

"You don't have to put your story on paper. This exercise shouldn't start by deciding you're wrong. Let me explain what happens to the pages. Then we can use a made-up example, or you can just listen."

Say truthfully whether pages are collected or kept.

Choose the next step

• Accepts another route: Use a made-up example, no writing. Invite one idea, or listening.

• Still declines: Clarify once and accept the answer. Talk through open participation questions separately, following program policy.

• Others also object: Give everyone the same options. Don't ask peers to persuade Tom.

• Access barrier: Offer an adaptation you can actually provide, and check that it helps.

• Current harm or clinical change: Take reported harm seriously; don't argue it away. Pause the exercise and use your support, assessment or emergency procedure.

Return to Check the Facts

Use an instructor moving an unfinished bowl. The made-up participant feels ashamed and anxious and expects rejection. Separate the action from the interpretation. Look at missing facts, other explanations, and evidence of exclusion. Consider likelihood, coping, and whether the emotions and their intensity fit what's known. Don't invent reassurance.

Support and record

With a co-facilitator, one steps out, one stays. Alone, stay with the group and call for staff if needed; never leave it on its own. Record his actual choices, adaptations, contributions and follow-up. Listening doesn't show mastery.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

A new member joins midway through the cycle

Scenario 5 · Adult IOP · Planned skill: DBT Problem Solving

Notice

Priya arrives on time but lacks context from earlier lessons. Lee wants application time. A lesson introduction doesn't replace essential admission or consent preparation.

First response

"Priya, I skipped the starting point. Let me show it. Lee, we'll keep time to put it to use too."

Choose the next step

• Finds an entry point: Acknowledge her specific contribution. One contribution doesn't mean she's fully oriented.

• Still unclear: Shrink it to one task, model it accessibly, and arrange teaching support. Watching is still an option.

• Returning member objects: Offer starting and application tasks to everyone. Don't rank members or ask peers to teach Priya.

• Essential preparation gap: Use the preparation process before sensitive participation. Get missing admission information from staff.

• Significant clinical concern: Use your assessment or emergency procedure; call for staff and stay with the group. Basic questions are different.

Return to Problem Solving

Use an appointment reminder with no address. Identify the situation, check the facts, set the goal, generate options, compare and choose one, try it, and review the result. Check whether they have a letter, phone, or other way to ask. If the call drops, find another way rather than blaming effort.

Support and record

With a co-facilitator, one explains individually, one stays. Alone, stay with the group and call for staff; never leave it on its own. Confirm who follows up and when. Record what she showed she understood and what was arranged, without assuming readiness.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

The facilitator makes a harmful assumption

Scenario 6 · Adult IOP · Planned skill: DBT Validation

Notice

You linked Andre's unfinished page with not caring about recovery. Correct it even if nobody objects. Quiet isn't repair.

First response

"Andre, I assumed an unfinished page meant you didn't care. That was unfair, and I'm sorry. You don't need to explain your circumstances."

Choose the next step

• Names the impact: Reflect accurately, own what you did, and change the invitation. Don't ask for forgiveness.

• Remains distrustful: Say what will change. Offer the real feedback process or another staff member, once you know they're free.

• A peer defends you: "Intention doesn't undo what I said. Andre doesn't need to agree that it was harmless."

• Goes quiet: Offer a check-in you can deliver, once. Don't ask for reassurance.

• Possible harm or medical concern: Use your assessment or emergency procedure; someone stays with the group. Criticism alone is different.

Return to Validation

Ask, "Was the task useful, or did something get in the way?" Nobody has to share personal circumstances. If the group can carry on, use a made-up friend upset about a canceled event. Listen and reflect; check you've got it right without inventing feelings or history. Validation isn't agreeing with every conclusion. Don't use Andre as the example or treat resuming as repair.

Support and record

With a co-facilitator, one steps out, one stays. Alone, stay and call for staff; never leave the group on its own. Promise only follow-up you can deliver. Seek supervision. Record your own conduct and Andre's response, including unresolved concerns.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

A member repeatedly interrupts

Scenario 7 · Adult IOP · Planned skill: DBT One Mindfully

Notice

Gloria starts talking before several members finish. Give turns back without guessing at intent or asking peers to manage Gloria.

First response

“Gloria, I’ll come to you next. I want to hear the end of this turn first.”

Ask the interrupted speaker whether they want to finish or pass. Keep the place in line you promised Gloria.

Choose the next step

• Uses the cue: Come to her when you said you would. A note is optional.

• Overlaps again: Restate the turn once, agree on a usable cue, and change the format if needed. Don’t debate whose idea came first.

• A peer criticizes: Manage turns yourself. Stop personal labels and give the interrupted speaker their choice back.

• Discussion remains unworkable: Pause the open exchange. Call for staff and follow your participation procedure, not automatic removal.

• Possible harm or clinical change: Stop teaching and use your assessment or emergency procedure. Don’t finish the exercise first.

Return to One Mindfully

Make sure the order is clear and interrupted members could finish or pass. Use a separate folder example: notice the mind jumping to a shopping list and return to the current step. Offer another task or watching. Silence isn’t mastery.

Support and record

With a co-facilitator, one steps out, one stays. Alone, stay and call for staff; never leave the group on its own. Record actual interruptions, cues, responses, and follow-up done or pending, leaving other members out.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

Advice takes over another member's turn

Scenario 8 · Adult IOP · Planned skill: DBT Validation

Notice

Nia gives directives before Miles asks for ideas. Others pile on, and Miles stops talking. Silence isn't consent. Miles owes nobody an explanation.

First response

"Let's pause the advice. Miles, do you want us to listen, or would you rather stop here?"

Choose the next step

• Wants listening: Invite an optional reflection of what Miles said. Check accuracy; no solutions.

• Advice continues: Name the limit once and stop the advice exchange. Don't debate whether quitting is wise.

• Wants ideas: Check what kind, invite brief experience, and let Miles stop it. Don't endorse instructions outside your role.

• Goes quiet or asks for support: Offer listening or a staff check-in you can deliver. No details needed.

• Possible harm or clinical change: Stop teaching. Use your assessment or emergency procedure. Don't finish the activity first.

Return to Validation

First stop the unrequested advice and handle any request for support. Then use a made-up neighbor disappointed that the community garden is full. Reflect what they said and check accuracy. Understanding isn't agreeing. Offer practice or watching.

Support and record

With a co-facilitator, one of you steps out and one stays. Alone, stay with the group and call for staff. Never leave the group on its own. Promise only follow-up that exists. Record what the member said and did, without naming peers, and whether follow-up was requested or confirmed.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

A member cannot use the worksheet

Scenario 9 · Adult PHP · Planned skill: DBT Describe

Notice

Luis says the page is unusable. Don't call it refusal or ask for a diagnosis, reading history, or public proof.

First response

"This page isn't working. I'll explain one step without writing. You can answer aloud, try privately, or watch."

Stop peers from looking at the page or taking over.

Choose the next step

• Spoken explanation helps: Model one step and invite an optional answer. Give feedback on what Luis showed.

• Still unclear: Change the explanation or format; don't just say it louder. Don't test Luis in public.

• Access still unresolved: Call for staff help and offer what you actually have. Watching a model Luis can't follow doesn't count.

• Sudden visual or cognitive change, possible harm, or medical concern: Stop teaching. Use your assessment or emergency procedure. Don't finish the example first.

Return to Describe

First confirm Luis has a way in or help is coming. Use a separate folded-towel example: describe the uneven edge instead of guessing at carelessness. Adapt the object or sense; no touching, reading, or writing required. A spoken detail can show one step; private practice stays unobserved.

Support and record

With a co-facilitator, one of you explains privately and one stays. Alone, stay with the group and call for help. Never leave the group on its own. PHP doesn't guarantee instant backup. Record stated needs, formats used, responses, and anything pending, without diagnosing a cause.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Access source: AHRQ, Health Literacy Universal Precautions Toolkit, Tool 5. AHRQ Tool 5

Standalone quick reference

A tired member struggles with the lesson

Scenario 10 · Adult PHP · Planned skill: DBT Observe

Notice

Karen says she's exhausted and can't manage several steps. Don't assume a cause. Answering a question doesn't rule out a clinical problem.

First response

"Karen, I hear that you're exhausted. Do you need staff help now, or would a simpler way to take part be useful?"

No health details in front of the group. Emergency action comes first.

Choose the next step

• Smaller task fits: Offer one step with no writing, or watching. Watch for changes.

• Feels unwell or asks for help: Get a clinical assessment promptly. No practice attempt needed first.

• Medication question: Connect her with the right clinician. Don't advise on doses.

• Peers judge effort: Stop the comparison; share nothing private.

• New confusion, hard to rouse, slurred speech, unsteadiness, or unusually slow breathing: Stop teaching and use your emergency procedure immediately. Don't finish the activity or wait for a break.

Return to Observe

Start any needed assessment and confirm support before teaching resumes. Choosing to listen isn't a reason to wait. Use a separate cabinet-light example, with another sense if needed; no closed eyes, breathing changes, or movement. Taking part doesn't make someone medically fit to continue.

Support and record

With a co-facilitator, one of you is with Karen and one stays. Alone, stay with the group and call for help. Never leave the group on its own. Escalate if help is slow. Confirm who has the handoff. Record what you saw, reported practice, requests, and anything pending.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Safety source: MedlinePlus, Confusion. MedlinePlus Confusion

Standalone quick reference

Silence after a painful disclosure

Scenario 11 · Adult IOP · Planned skill: DBT Validation

Notice

Rachel shares a recent loss, then reads the silence as rejection. Don't assume what the silence means or press peers to react.

First response

"Rachel, I'm sorry to hear about your brother. You don't need to make this easier for us. Would you like us to listen, or stop here?"

Choose the next step

• Wants acknowledgment: Reflect what it means to her; check accuracy. Stop when asked.

• Feels rejected: Acknowledge the impact; don't speak for everyone. Invite a response only if she wants one.

• Peers explain or compare losses: Stop the explanations. Nobody has to share their own story.

• Asks for support: Confirm who can see her and when. Promise only what's available.

• Possible harm or clinical change: Stop teaching. Use your assessment or emergency procedure. Don't finish the reflection first.

Return to Validation

First acknowledge the disclosure and handle any urgent support need. Use a separate canceled-choir example. Reflect what was said, check accuracy, and offer practice or watching. Rachel's grief is not the demonstration.

Support and record

With a co-facilitator, one of you steps out and one stays. Alone, stay with the group and call for staff. Never leave the group on its own. No private details needed. Record her preference, what you did, and whether contact is confirmed or pending.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Standalone quick reference

Language support does not meet the task

Scenario 12 · Adult PHP · Planned skill: DBT Describe

Notice

The interpreter connection fails. Rosa says she doesn't understand. Some everyday English, or a willing peer, isn't clinical interpretation.

First response

Through a working communication route: "The connection failed. I need to fix how the instructions reach you. You don't need to explain personal information to the group." If nothing qualified works, say only that you're getting the interpreter back. No clinical instructions, no peer interpreting.

Choose the next step

- Qualified support reconnects: Confirm the setup, speak to Rosa directly in short segments, and check one instruction.
- Still unusable: Call the backup language service. Watching a model Rosa can't understand isn't access.
- Peer volunteers: Thank them. Use your qualified language service; never have a patient interpret.
- Rosa prefers peer help: Acknowledge it and check your program's access process. Don't improvise an exception.
- Possible harm or sudden clinical change: Use your assessment or emergency procedure now, with language support alongside. Don't wait for the worksheet or the connection.

Return to Describe

Confirm working communication or another qualified option. Use a separate envelope example: describe the outside without guessing what's inside. Offer supported practice or an example Rosa can follow. No private material.

Support and record

With a co-facilitator, one of you sorts out access and one stays with the group. Alone, stay with the group and call for help. Never leave the group on its own. Record the language support used, responses, unresolved limits, and who owns the handoff.

Sources: Linehan (2015), mindfulness teaching notes; AHRQ, Tool 9. AHRQ language access

Standalone quick reference

Check-in has used the practice time

Scenario 13 · Adult IOP · Planned skill: DBT One Mindfully

Notice

Twelve minutes remain. Own the timing and change the plan without blaming the last speaker or racing through every slide.

First response

“I let check-in run long. We have twelve minutes left. I’ll focus on one useful practice and leave time to close.”

Choose the next step

• Smaller goal fits: Name one step, offer brief practice or watching, and keep time to close.

• Member objects: Acknowledge the lost practice and offer a change you can make. Don’t defend the whole session.

• Extra topic can wait: Confirm a real option and what’s pending. Don’t invent follow-up.

• Possible urgent need (for example, “I don’t know whether I can get through tonight”): Thank them, get staff now, and ask them to stay with you until staff connect. No questioning in front of the group. If you need an immediate safety answer, ask quietly; don’t wait for a private room. Fuller assessment happens privately. The session ending is no reason to wait. Don’t finish practice first.

Return to One Mindfully

Deal with immediate needs and say what you’re leaving out. Use a separate library-announcement example: notice the drift to meal planning and return to the announcement. Offer another format if needed. Writing and personal sharing are optional. Watching isn’t confirmed practice.

Support and record

With a co-facilitator, one of you steps out and one stays. Alone, stay with the group and call for staff. Never leave the group on its own. Confirm who has the handoff. Record what was actually covered and reported, and anything pending, not the agenda.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A comment targets identity

Scenario 14 · Adult IOP · Planned skill: DBT Validation

Notice

Terry dismisses same-sex families after Keisha mentions a partner. Stop the comment. Keisha's objection isn't an equal harm.

First response

"Keisha, you don't need to defend your family. Terry, that comment dismissed same-sex families, and I'm stopping it."

Choose the next step

- Statement withdrawn: Offer Keisha listening or support you can deliver. No response, thanks, or forgiveness is owed.
- Belief debate begins: Restate the limit once. Don't debate whose identity or family is legitimate.
- Comment repeats: Pause teaching and call for staff. Don't improvise expulsion.
- Peers defend intent: Return to the behavior and to support. Nobody has to disclose their identity.
- Threat, possible harm, or clinical change: Use your assessment or emergency procedure. Don't finish the skill exercise first.

Return to Validation

Stop the comment, acknowledge its impact, and answer any support request before teaching. Use a separate overlooked-volunteer example. Reflect what the person says and check you've got it right. Terry doesn't practice on Keisha's experience.

Support and record

With a co-facilitator, one steps out and one stays. Alone, stay with the group and call for staff. Never leave the group on its own. Confirm who follows up. Record the member's own behavior and actual responses, without naming peers. Taking a comment back doesn't mean the harm is repaired.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Standalone quick reference

Laughter interrupts a contribution

Scenario 15 · Adult PHP · Planned skill: DBT Check the Facts

Notice

Jamal stops talking after two members laugh. The interruption is clear; who it was aimed at isn't. Take the impact seriously.

First response

"Jamal, the laughter interrupted you. You don't have to continue your example."

"Please stop the side conversation. We need one speaker."

Choose the next step

• Aside stops: Offer finishing, passing, listening, or support you can deliver. No explanation or apology has to be accepted.

• Intent is disputed: Hear a brief account. Don't debate Jamal's feelings or ask to see private messages.

• Disruption continues: Pause teaching and use your conduct and support procedures.

• Member wants to leave: Say you're asking for support. Don't block or follow. Stay with the group and use the Scenario 29 steps.

• Threat, urgent harm, or clinical change: Use your assessment or emergency procedure. Don't finish the worksheet first.

Return to Check the Facts

Go on once the interruption has stopped and support requests have a workable answer. Use a separate rain-forecast example: tell information from prediction, name the gardener's anxiety, and ask whether the feeling and its strength fit the facts. Never use the skill to argue away real harm.

Support and record

With a co-facilitator, one steps out and one stays. Alone, stay with the group and call for staff. Never leave the group on its own. Record Jamal's words, choices, and actual responses, without naming peers. Confirm who owns follow-up. A quiet room hasn't settled anything.

Skill source: Linehan (2015), emotion-regulation teaching notes. Guilford teaching notes

Standalone quick reference

Members question a boundary's fairness

Scenario 16 · Adult IOP · Planned skill: DBT DEAR MAN

Notice

Devon questions check-in timing and others join in. A fair concern and crosstalk can happen together.

First response

"You're asking whether I used the same time limit for everyone. I'll hear one concern at a time."

Choose the next step

- Clear error: Own it and offer a fix you can deliver. Don't promise time you don't have.
- Unresolved comparison: Clarify the next turns and the review step. Confirm who follows up and when.
- Overlapping speech: Restore one speaker. If disruption continues, pause teaching and call for staff.
- Private comparisons requested: Explain the general approach. Don't discuss another member's circumstances.
- Threat, urgent harm, or clinical change: Use your assessment or emergency procedure. Don't finish the exercise first.

Return to DEAR MAN

Go on once the limit is clear and the concern is answered or under review. Use a separate choir member's request for written rehearsal details to practice Describe, Assert, Reinforce. A complaint doesn't need a polished request to be heard.

Support and record

With a co-facilitator, one steps out and one stays. Alone, stay with the group and call for staff. Never leave the group on its own. Don't argue with each angry statement or demand private explanations. Record Devon's concern, any correction, and pending review, without naming peers. Silence isn't agreement.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Standalone quick reference

Peers ask for graphic details

Scenario 17 · Adult IOP · Planned skill: DBT STOP

Notice

Maureen mentions a painful event; peers ask for details. Stop the pressure without implying it can't be acknowledged.

First response

"Maureen, you don't need to describe what happened for us to take your experience seriously."

"Please stop asking for details. Maureen doesn't owe the group that account."

Choose the next step

• Wants to listen: Respect that and agree on a support signal she can use.

• Wants a full account now: Explain the format limit and arrange individual support. Don't promise a treatment outcome.

• Peers persist: Restate the limit once. No debate, required reassurance, or round of shared trauma.

• Concerning clinical change or urgent harm: Use your assessment or emergency procedure. Don't assume grounding is enough or finish the worksheet first.

Return to STOP

Go on once the questions stop and current support needs have a workable answer. Use a separate ticket-machine frustration. A step back can be a pause in place. Don't make the event she mentioned the practice task.

Support and record

With a co-facilitator, one steps out and one stays. Alone, stay with the group and call for staff. Never leave the group on its own. A sent message isn't a handoff: confirm a named person has it. Record actual choices and actions without unnecessary trauma details or peer identities.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

Diagnosis or medication advice is requested

Scenario 18 · Adult PHP · Planned skill: DBT DEAR MAN

Notice

Walter feels “wired” after a medication change and asks about diagnosis and dosing. Treat it as a clinical concern, not an off-topic question.

First response

“That change matters. I can’t decide a diagnosis or a dose here. I’m contacting clinical staff now.”

Choose the next step

• Help available: Set up an individual review; confirm a named clinician has it. Keep details private.

• Usual contact unavailable: Use the backup clinical contact and escalation plan, not a routine appointment. Before Walter leaves, confirm who will answer the dose question tonight and how to reach them.

• Peers give instructions: Stop advice about changing or sharing medication. Don’t debate treatments.

• Significant clinical change or possible urgent harm: Use your assessment or emergency procedure. Don’t finish a worksheet or test the skill first.

Return to DEAR MAN

Go on once the clinical response is under way and someone is with the group. Use a separate request for a written repair estimate. Clinical attention never depends on a well-worded request. This card gives no instruction to take, skip, or change a dose.

Support and record

With a co-facilitator, one steps out and one stays. Alone, stay with the group and call for staff. Never leave the group on its own. Record Walter’s report, who you contacted, and whether the handoff is confirmed or pending, without peer histories. Don’t infer cause or diagnosis.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Standalone quick reference

A guarantee is requested

Scenario 19 · Adult IOP · Planned skill: DBT Pros and Cons

Notice

Hannah asks for guaranteed relief; a peer credits effort. Answer honestly and stop the pressure.

First response

“You’re looking for something that will help tonight. I can’t promise how you’ll feel after using this.”

“We won’t judge another person’s effort or guarantee their result.”

Choose the next step

• Chooses to watch: Accept it. No belief or endorsement required.

• Earlier claim was overstated: Correct it directly. Don’t swap in another guarantee.

• Demands certainty again: Restate the limit briefly and offer the task or support. Don’t debate each objection.

• Practical need identified: Connect her with staff who can help. The worksheet is never a condition for help.

• Urgent harm or clinical change: Use your assessment or emergency procedure. Don’t finish the exercise first.

Return to Pros and Cons

Go on after clarifying what the exercise is for and answering current support needs. Use a separate parcel-complaint example to weigh short- and long-term consequences. No private history needed, and no promise of instant relief.

Support and record

With a co-facilitator, one steps out and one stays. Alone, stay with the group and call for staff. Never leave the group on its own. Record her actual choices, her report of benefit or no benefit, and confirmed or pending follow-up, without naming peers.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

“We’ve done this already”

Scenario 20 · Adult IOP · Planned skill: DBT Effectively

Notice

Returning members question the repetition; a newcomer closes the handout. Don’t rank members by experience.

First response

“You’ve seen this before. Let’s make the practice worth your time and keep the explanation there for everyone.”

Choose the next step

- Wants more challenge: Offer a made-up example with competing goals and a clear end.
- Questions the schedule: Say what you know, fix real mistakes, keep any promised review.
- Starts teaching peers: One speaker. No compulsory teaching, comparing, or debating who knows more.
- Needs the basics: Keep the explanation open to all. Nobody has to say how long they’ve attended.
- Urgent harm or clinical change: Use your assessment or emergency procedure now. Don’t finish the task.

Return to Effectively

Resume with one shared aim and two supervised options. Use the chair examples to compare actions against a chosen goal. Effective isn’t obedient or guaranteed. Watching is allowed.

Support and record

With a co-facilitator, one leads, one supports the harder task. Alone, keep both options in the room. Never leave members or a subgroup alone; call for staff for individual support. Record what each member did and any confirmed or pending review. Leave other members out. Knowing isn’t mastery.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

Arrivals and departures interrupt teaching

Scenario 21 · Adult PHP · Planned skill: DBT STOP

Notice

Dana arrives mid-demonstration as another member is called out, and recaps start. Fix coordination and access; blame nobody.

First response

“Welcome, Dana. Have a seat. No need to explain anything.”

“Let’s hold the recaps. I’ll give the current instruction, and we won’t repeat personal examples.”

Choose the next step

- Brief entry: Offer watching or the next made-up example, and a usable prompt.
- Scheduled departure: Confirm which staff member is receiving the member, and how.
- Repeated traffic: Pause teaching and get staff help. Don’t restart private check-ins.
- Criticism begins: Hear the scheduling concern. Don’t ask for private reasons or debate accommodations.
- Urgent harm or clinical change: Use your assessment or emergency procedure. Don’t finish the worksheet first.

Return to STOP

Go on once transitions have a responsible person, everyone has the task, and someone can stay with the group. Use the made-up wrong-bulb example. A step back can be a pause in place. Nobody’s arrival or appointment becomes practice material.

Support and record

With a co-facilitator, one handles transitions, one stays. Alone, stay with the group and call for help. Never leave it alone or send a peer as escort. Record Dana’s actual arrival and participation; leave other members out. Don’t imply full attendance or a completed assessment.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

Membership changes unexpectedly

Scenario 22 · Adult PHP · Planned skill: DBT Validation

Notice

Rory's usual practice partner is absent, and peers are guessing why. This covers membership uncertainty, not a walkout.

First response

"You were expecting to practice together. I can't share someone else's care or plans, but I can explain today's options and how to get support."

Choose the next step

- Needs a task option: Offer watching, a made-up demo, or a pairing you can supervise. No replacement partner needed.
- Questions continue: Restate the privacy limit briefly. Don't debate it or hint at a reason.
- Outreach proposed: No organized contact, shared details, or peer investigating. State the program's actual rules.
- Wants support: Call staff and confirm who has it.
- Urgent harm or clinical change: Use your assessment or emergency procedure. Don't finish an exercise first.

Return to Validation

Go on once guessing stops, practice plans are clear, and support needs are met. Use a separate changed-walking-routine example. Nobody has to share a loss, guess motives, or announce closure.

Support and record

With a co-facilitator, one arranges support and one stays with the group. Alone, stay with the group and call for help. Never leave it alone. Record what Rory said and chose. Don't name the absent member or guess about their care. A sent request isn't a handoff.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Standalone quick reference

Practice increases distress

Scenario 23 · Adult IOP · Planned skill: DBT Observe

Notice

Mei says body focus makes things worse and asks to stop. Stop first; explain later, if at all.

First response

“Yes, we’re stopping. You don’t need to keep focusing on your chest.”

“Please don’t coach Mei to keep going.”

Choose the next step

• Wants to listen: Let Mei listen. No other exercise, no explanation needed.

• Wants an alternative: Offer a short neutral outside focus only if Mei agrees. Stop if it’s unwanted or unhelpful. No endurance goal.

• Chest pain, trouble breathing, or feeling faint: Treat as a possible medical emergency and use your emergency procedure now.

• Wants support: Call staff and confirm who has it.

• Significant clinical change or urgent harm: Use your assessment or emergency procedure. Don’t assume panic or dissociation, finish the worksheet, or require grounding first.

Return to Observe

Go on only after the original practice ends and current needs are handled. Demonstrate noticing a clothespin, with no touching or inward focus required. Don’t promise calm. Watching or declining is fine.

Support and record

With a co-facilitator, one handles support and one stays with the group. Alone, stay with the group and call for help. Never leave it alone or ask peers to watch Mei. Record Mei’s report, request to stop, choices, and actual or pending contact. No guessed diagnoses or peer histories. Quiet isn’t an assessment or proof of improvement.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

The room is difficult to tolerate

Scenario 24 · Adult PHP · Planned skill: DBT Observe

Notice

Screen flicker and corridor noise get in the way. Fix the room before turning discomfort into practice.

First response

“I’m pausing the instructions while we fix what we can. You don’t need to explain a diagnosis.”

Choose the next step

- Controllable barrier: Reduce it and give the prompt in a usable form. Check what remains.
- Room still unsuitable: Pause teaching; call for staff. Confirm the room, access, and supervision before moving.
- Different access needs: Look at concrete options. No vote on whose need is real, no required medical disclosure.
- Peers say tough it out: Stop the pressure. Don’t debate normal tolerance.
- Clinical change, urgent harm, or environmental emergency: Use your assessment or emergency procedure, not the worksheet. Closing a door is fine; never lock or block an exit.

Return to Observe

Go on once the task is usable and support needs are handled. Use the paper windmill with a spoken description. No touching, closed eyes, or attending to noise required. No promise of relief.

Support and record

With a co-facilitator, one stays and one sorts out the room. Alone, stay with the group and call for help; never leave it alone to find another room. Record Sage’s report, what changed, what was still hard, and what’s pending. No guessed diagnoses or peer histories.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A suggestion does not fit real constraints

Scenario 25 · Adult PHP · Planned skill: DBT Problem Solving

Notice

Your home-practice idea assumed private time and space Carlos lacks. Acknowledge the mismatch before more advice.

First response

“I suggested quiet time you don’t have. That plan doesn’t fit. Would trying a step here help, or would you prefer our made-up example?”

“We can use a made-up example. You don’t need to explain your circumstances.”

Choose the next step

• Wants adaptation: Offer a step he can do in session. A shorter home task doesn’t fix the barrier.

• Declines public brainstorming: Stop the advice and respect the choice. It isn’t resistance.

• Requests resources: Call the right staff contact. Check availability before promising.

• Peers insist: Ideas need his permission. No debate, proof of finances, or group vote.

• Current harm or clinical change: Use your assessment or emergency procedure, not the worksheet. A coping tweak doesn’t replace a response to an unsafe situation.

Return to Problem Solving

Go on once the assumption is corrected and support needs are handled. Use the seed-exchange label problem, constraints included. Nobody has to share housing, family, or money details.

Support and record

With a co-facilitator, one arranges support and one stays with the group. Alone, stay with the group and call for help; never leave it alone to find resources. Record the barrier, Carlos’s choice, and confirmed or pending contact. Leave other members out. A written plan isn’t proof it’s doable.

Skill source: Linehan (2015), emotion-regulation teaching notes. Guilford teaching notes

Standalone quick reference

A legitimate concern about the intervention

Scenario 26 · Adult IOP · Planned skill: DBT Check the Facts

Notice

Remy is right: you called it a distortion before hearing what happened. Answer that before defending the method.

First response

“You’re right. I called it a distortion before I’d heard what happened. I shouldn’t have started there. We can leave your example out.”

Choose the next step

• Wants a made-up example: Use a truly separate one, not Remy’s story in disguise. Watching or passing is fine.

• Asks for review: Explain how your program takes complaints and who follows up. No skill practice required first.

• Stays angry: Hear the concern and say briefly what happens next. Don’t argue every point or ask for reassurance.

• Current harm or clinical change: Take it seriously. Stop and get a clinician or use your emergency procedure. Don’t argue it away or finish the worksheet first.

Return to Check the Facts

Go on once the framing is fixed and any review or clinical need is handled. Use the made-up ferry timetable: name the traveler’s worry, check the current departure, and ask whether the feeling and its strength fit the facts. Checking can show a worry is accurate.

Support and record

With a co-facilitator, one steps out and one stays. Alone, stay with the group and call for staff. Never leave the group on its own. Record what you did, Remy’s response, and any review requested. Leave other members out. An apology starts the repair; it doesn’t finish it.

Skill source: Linehan (2015), emotion-regulation teaching notes. Guilford teaching notes

Standalone quick reference

The facilitator applies a boundary inconsistently

Scenario 27 · Adult IOP · Planned skill: DBT DEAR MAN

Notice

You checked a personal notification after asking members to put phones away. Logan has named a real inconsistency.

First response

“You’re right. I checked a personal notification while you were talking. I’m putting the phone away. Would you like to finish?”

Choose the next step

• Wants to continue: Give the turn back. Don’t ask for acceptance, thanks, or reassurance.

• Reports a pattern: Hear one brief point and explain how review works. Don’t argue over past events.

• Peers defend staff: Stop the pressure. Nobody needs to defend or forgive you.

• Private exceptions questioned: Explain how to request one without naming anyone’s arrangements.

• Urgent harm or clinical change: Stop and get a clinician or use your emergency procedure. Don’t finish the worksheet first.

Return to DEAR MAN

Go on once the behavior is fixed and any review or clinical need is handled. Use a made-up history-club request for slides in advance. A complaint doesn’t need a skill formula to be heard, and Logan needn’t rehearse it aloud.

Support and record

With a co-facilitator, one steps out and one stays. Alone, stay with the group and call for staff. Never leave the group on its own. Record what you did, Logan’s response, and any review requested. Leave other members out. Joining back in doesn’t mean trust is back.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Standalone quick reference

Staff give conflicting directions

Scenario 28 · Adult PHP · Planned skill: DBT Effectively

Notice

One facilitator allows a made-up example or watching; the other requires a personal example. Ren asks which applies. Staff sort it out.

First response

“We gave you two different instructions. That’s ours to sort out. We’re pausing, and nobody needs to share a personal example while we do.”

Choose the next step

• You can agree: Give one instruction, the choices, and who leads. Don’t pretend the directions were the same.

• You still disagree: Keep the exercise paused and call your supervisor or staff support. No public debate or member vote.

• Worried about sharing: Nobody shares private material to settle this. Explain this task only; no promises about consequences.

• Clinical concern raised: Stop and use your assessment or emergency procedure. A made-up example doesn’t replace a clinical response. Don’t finish the worksheet first.

Return to Effectively

Go on only once roles and the instruction are clear and current needs are handled. Use a separate display-table teamwork example. Members don’t act out staff or referee the real disagreement.

Support and record

With two facilitators, agree who stays before you confer; never both leave. Alone, stay with the group and call for help. Never leave it on its own or send a member to settle it. Record Ren’s question, what was clarified, and what’s pending. Leave other members out. Confusion isn’t refusing treatment.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A member leaves during tension

Scenario 29 · Adult IOP · Planned skill: DBT STOP

Notice

Brianna walks out; you don't know where, or whether staff have made contact. No threat was made. A walkout alone doesn't show safety or danger.

First response

Pause teaching and follow your procedure for a member who leaves, right away.

"Please stay here with me while I get staff. Nobody needs to follow."

Choose the next step

• Alone: Stay with the group; never leave it to search. Tell staff what you saw and where Brianna was last seen.

• Co-facilitator present: Say who stays and who responds. Never both follow.

• No answer: Use your backup or escalation contact. Keep going until a named person confirms what they'll do and when.

• Contact confirmed: Check what they've taken on. Contact isn't an assessment and doesn't clear anyone to return.

• Known urgent risk, possible immediate harm, or clinical change: Use your assessment or emergency procedure now. Don't wait for a return or finish the worksheet.

Return to STOP

Go on only once a named person is handling it, clinical needs are met, and the group is covered. Otherwise stay paused and escalate. Use a separate cabinet-painting example. Stepping back can happen in place; it doesn't mean leaving.

Support and record

No peer search, private history, or debate about motives. Don't block the door. Record what was said, who you contacted, and what's still unknown. Don't record safety or a decision nobody assessed.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

Returning after individual support

Scenario 30 · Adult PHP · Planned skill: DBT Describe

Notice

Micah returns with staff; peers ask for details and an apology. Being back doesn't mean a care plan is in place.

First response

"You don't need to explain the individual conversation. We're pausing while staff confirm the plan for coming back."

Choose the next step

• Plan confirmed: Privately confirm what you're taking on, support needed, and participation choices.

• Plan unclear: Keep teaching paused and ask staff. Micah's "I'm fine" isn't a handoff.

• Peers demand repair: Stop the questions. No required apology or forgiveness. Explain how to raise a concern; don't discuss Micah's care.

• Conduct resumes: State the current limit and use your program's procedure.

• New clinical change or urgent harm: Stop and use your assessment or emergency procedure. No worksheet or grounding first.

Return to Describe

Go on only when the plan is clear and current needs are met. Use a separate market-stall layout, described aloud if needed. Don't make Micah's behavior the example or the practice a readiness test.

Support and record

With a co-facilitator, one steps out and one stays. Alone, stay with the group and call for staff; never leave it for a handoff talk. Record the plan, who took responsibility, and what Micah chose. Leave other members out. Quiet or an apology doesn't mean safe or repaired.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A disruption recurs across sessions

Scenario 31 · Adult IOP · Planned skill: DBT Problem Solving

Notice

Kevin keeps giving advice during others' turns. Handle this one and set up a review. Don't guess at intent.

First response

"Kevin, please hold the idea while the speaker finishes. The speaker keeps the turn."

Choose the next step

• Stops: Give the turn back. Offer a doable way to hold the idea; don't promise time for every one.

• Pattern persists: Privately review what you've seen, your instructions, barriers, and Kevin's view. Agree on a cue, an alternative, and a review date.

• Keeps interrupting now: Pause teaching and call for staff. No public argument or invented penalties.

• Peers complain: Hear the effect on them. Don't make them monitors or share Kevin's plan.

• Urgent harm or clinical change: Stop and use your assessment or emergency procedure. Don't finish the worksheet or wait on the cue.

Return to Problem Solving

Go on once the turn is protected and urgent needs are met. Use a separate repair-café tool-tracking problem. Members don't analyze Kevin or design consequences. A trial isn't a guaranteed fix.

Support and record

With a co-facilitator, one steps out and one stays. Alone, stay with the group and call for staff; never leave it for a private talk. Kevin needn't explain personal history. Record Kevin's behavior, any agreed cue, the response, and pending review. No peer names or labels.

Skill source: Linehan (2015), emotion-regulation teaching notes. Guilford teaching notes

Standalone quick reference

Ending while tension remains

Scenario 32 · Adult PHP · Planned skill: DBT STOP

Notice

Four teaching minutes remain. Emery says the concern is unresolved. The lesson can end; care and supervision continue.

First response

“We’re near the end, and this still needs attention. I’m checking who can hear your concern next.”

Choose the next step

• Nonurgent follow-up accepted: Tell Emery who will follow up and when. Don’t promise agreement or resolution.

• Follow-up not confirmed: Stay with the group; try your backup contact or escalate. The hour ending doesn’t end your job.

• Debate restarts: Name the limit briefly. Nobody has to apologize, forgive, share private details, or find a positive takeaway.

• Appointment or transport: Arrange it with the right staff. This card doesn’t authorize holding or releasing anyone.

• Urgent harm or clinical change: Stop closing. Use your assessment or emergency procedure now. Don’t finish a worksheet, require a skill, or wait for an unaccepted handoff.

Return to STOP

Only if care and handovers allow. Use a made-up mislabeled-photo-album example. Stepping back can happen in your seat. It isn’t a readiness or discharge test.

Support and record

With a co-facilitator, one takes the group, one handles follow-up. Alone, stay with the group and call for help; never leave it on its own. Record the concern, accepted or pending follow-up, and next actions. Omit unrelated peer details and unsupported safety conclusions.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

A member says they plan to end their life

Scenario 33 · Adult IOP · Planned skill: DBT Self-Soothe

Notice

Daniel says what he intends, when, and how. Stop the lesson. An apology or “I was joking” doesn’t lower the urgency.

First response

“Daniel, thank you for telling me. I’m taking this seriously, and I’m getting a clinician here now. I’d like you to stay with me until they arrive.”

Choose the next step

• Urgent risk statement: Call for a clinician now with Daniel’s exact words. Stay with Daniel and the group. No answer? Call your backup at once.

• Takes it back: Keep the response going. Don’t debate or delay the clinician.

• Stands to leave: Ask him to stay. Don’t block or follow. Tell staff his words, direction, and description. Follow your emergency procedure.

• Says he already took something: Medical emergency. Use your emergency procedure now.

• Peers react: Say you’ve called for help; say a clinician has him only once confirmed. Respond to others’ requests privately.

Return to Self-Soothe

Only after a clinician has taken over and someone is with the group. You can end early. If you continue, keep it short, optional and made-up; close with how to reach staff today.

Support and record

Never leave the group alone or send a member for help. Don’t ask about the plan publicly or promise secrecy. If you need one immediate answer (has he taken anything, are the pills on him), ask quietly; the clinician does the assessment. Record exact words, times, actions, and accepted or pending responsibility. A sent message isn’t a handoff: a named person confirms taking it, what they’ll do, when, and who stays with the group.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

A member appears intoxicated

Scenario 34 · Adult PHP · Planned skill: DBT Observe

Notice

Shannon has slurred speech, unsteadiness, and a smell of alcohol. Other medical causes are possible. New or sudden speech or balance changes, reduced responsiveness, or abnormal breathing mean emergency response now.

First response

“Shannon, I’m concerned about your speech and balance. Please stay here while I get medical help now.”

Choose the next step

• Accepts help: Report what you saw and onset, if known. Get a named person to confirm they’re coming. No answer? Use your backup.

• Loud or argumentative: Don’t debate. Keep your distance. “I’m getting someone to check on you.” Don’t block the door.

• Less responsive, vomiting, or abnormal breathing: Start your emergency medical response now. Keep the group supervised; call another staff member to take the group if you need to attend to Shannon.

• Mentions driving: Tell responding staff right away; program procedure decides transport.

• Peers comment: “I’m getting staff support.” Say staff have taken over only once confirmed. Keep supervising the group.

Return to Observe

Only after a clinician has taken responsibility. Use a neutral object, such as a glass of water. Keep Shannon and the interruption out of it.

Support and record

Stay with Shannon and the group; never send a member for help. Record what you saw, exact words, times, accepted or pending responsibility, and transport concerns. Don’t infer intoxication or a diagnosis from appearance alone.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A member threatens another member

Scenario 35 · Adult IOP · Planned skill: DBT STOP

Notice

Cameron threatens Victor and steps toward him. Safety situation: stop the lesson.

First response

“Cameron, please give Victor space. I’m pausing the group and getting help now.”

Choose the next step

• Sits down: Thank him briefly. Call for help anyway. Keep them apart.

• Continues or moves closer: Emergency procedure or panic alert now. Move others away calmly. Don’t step between people or touch anyone.

• Physical aggression or weapon: Emergency. Follow your procedure, including 911 if that’s the local instruction. Help others leave if safe.

• Victor wants to leave: Support it, tell staff, and don’t send him alone or with a peer.

• Peers react: Give the spacing or move-to-safety direction the danger calls for; don’t insist on staying seated.

Return to STOP

Only after staff have taken over, someone is with the group, and the room is safe. Pausing or stopping is fine. If you teach, use a made-up irritated-email example.

Support and record

Keep exits clear; no restraint or blocking. Record the threat’s exact words, times, actions, and accepted or pending responsibility. Tell the clinical lead right away; protective duties vary by location.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

A virtual member hints at danger, then disconnects

Scenario 36 · Adult IOP · Planned skill: DBT STOP

Notice

Lisa says, “I’m not sure I’ll still be here tomorrow,” then leaves the meeting. Treat it as urgent until you know otherwise.

First response

“We’re pausing here. I’m staying with you while I arrange help. If you need support too, let me know.” Alert staff now, not after a callback.

Choose the next step

• Reached: The staff member calling gets a clinician to her on a private line and confirms where she is. You stay with the group.

• No answer: Alert staff now with Lisa’s words, the time, and her confirmed location. Follow the emergency procedure.

• Rejoins: Arrange clinical contact now, not after group. Confirm who has it and who has the group.

• Members want to help: “Please don’t message Lisa yourselves. I’m working with staff on reaching her.”

Return to STOP

Only once a named staff member is reaching Lisa and you can focus on the group. Use the printer-jam example, or stay paused.

Support and record

Stay with the group; being on camera isn’t enough. Confirm location and backup number every session. No member contacts Lisa. If staff don’t answer, use the backup or emergency route. Record her exact words, times, and who took responsibility (or that it’s pending).

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

Someone else is in a virtual member's room

Scenario 37 · Adult IOP · Planned skill: DBT Describe

Notice

Someone else is heard in Omar's room. Group privacy and Omar's safety are at stake.

First response

"Let's pause sharing for a minute while we check connections and privacy." Don't name the person or ask Omar to explain.

Choose the next step

• Privacy restored: Check nobody else can hear the group, see the screen, or overhear Omar. Headphones cover only the first.

• No privacy: Keep sharing paused; arrange another way to attend. Muting doesn't stop his room hearing or seeing the group.

• Possible danger: Use the agreed signal or a safe channel; private chat may be seen. Start the emergency response with his confirmed location.

• Other person intervenes: Use host controls and call for staff. Don't confront them. Arrange safe contact before cutting Omar off.

Return to Describe

Resume personal sharing only when privacy is back and any safety concern is with a named staff member. A made-up kitchen-counter task fills the pause but doesn't fix exposure.

Support and record

Stay with the group. Don't probe private history in group; never send members to check on Omar. Act on urgent danger before the exercise; if staff don't answer, use your backup. Record observations, privacy changes, contacts made, and what's open.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A member joins while driving

Scenario 38 · Adult IOP · Planned skill: DBT One Mindfully

Notice

Patricia joins from a moving car. Safety and privacy come first.

First response

“Patricia, please stop joining in while you’re driving. Come back once you’re parked somewhere safe and private. Don’t type or touch your phone while you drive.”

Choose the next step

• Rejoins: Confirm she’s parked safely somewhere private before she takes part. Apply the real attendance rules; don’t promise credit.

• Stays on: Remove her with the usual host controls. Don’t argue or ask for a typed reply while she drives.

• No other option: Pass the barrier to the team for a realistic later plan. Promise nothing; she still can’t take part while driving.

• Camera off, driving not confirmed: Use the agreed private check and access plan; camera off proves neither driving nor danger. Never ask a driver to type. A worrying statement or lost contact: use Scenario 36.

• Peers comment: Say briefly you’re handling it. Don’t say Patricia is safe.

Return to One Mindfully

Resume once she’s no longer taking part while driving and any separate safety concern is handled. Use the made-up cooking-and-phone example; don’t make Patricia the lesson.

Support and record

Stay with the group. No members monitoring her, no questions about private history, no public argument. Urgent concerns come before teaching. Record what she said, times, the host action, how reentry was confirmed, and pending follow-up. A frozen image or stalled traffic doesn’t prove she’s safely parked.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A private chat message asks you to keep a secret

Scenario 39 · Adult IOP · Planned skill: DBT Validation

Notice

Brooke writes that home doesn't feel safe and asks for secrecy. Reply quickly and safely; looking calm doesn't settle it.

First response

"Thank you for telling me. Can you safely use this chat now?" Use the agreed signal if any; ask only whether she's in danger now. Don't hold urgent action for a reply.

Choose the next step

• Immediate danger: Urgent response now, with her location and message. Confirm who has it, who has the group, and safe contact; escalate if unanswered.

• Denies immediate danger: Arrange prompt clinical follow-up and safe contact. A yes-or-no isn't an assessment. Promise only confirmed arrangements.

• Secrecy requested: Explain the real limits; share only what's relevant with the clinical team. Never promise secrecy.

• Leaves or stops replying: Use Scenario 36; tell staff her device may be monitored. Stop exposed messages; call back only if the safe-contact plan allows.

Return to Validation

Resume only once a named staff member has her response and safe contact in hand, and you can focus on the group. A sent message isn't a handoff. Until then, stay paused; then use a made-up example.

Support and record

Never read private messages aloud, send members to check on her, or leave the group. Unanswered? Use your backup. Record her exact words, actions, times, who took responsibility (or that it's pending), and the safe-contact plan.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Handoff and group reset

A one-page reminder to adapt to your program. Keep client information in the clinical record, not on this reusable page.

Make the request concrete

“This is [name and role] in [group and room]. I saw [what happened] and heard [exact words]. I need [what you need] now. I’m staying with the group. Can you confirm who is taking this on?”

Close the loop

You’re done handing off when a named person has told you they’re taking responsibility, what they’ll do, and when. If nobody answers, use your backup or emergency route. A sent message, the person coming back to group, or a quiet room doesn’t count.

If the room can pause safely

Give one short direction without sharing anyone’s private information. Offer quiet watching or a simple optional task, but only if it doesn’t take your attention away from the person who needs it. If safety, staffing, or privacy rules out teaching, keep the group in a supervised pause or move to your planned transition.

Before you resume or end

Check who still needs something, who has taken responsibility, and whether anyone else asked for support. Be honest about what isn’t settled yet. Don’t promise availability you can’t confirm.

Record and review

Write what you saw, the exact words that mattered, what you did, how the person responded, the times you made contact, who accepted responsibility, and what is still pending. Use each person’s own record and leave other members out, unless something specific is needed for safety or required by law. After an acute event, use your program’s incident and supervision process. Don’t record guesses as findings.

Afterward

Once the immediate event is handled, check what members and staff need. Volume 1, SP-CD offers an optional support session for the group. Nobody should be asked to retell what happened, and teaching doesn’t have to resume just because the schedule says so. Talk it through with your supervisor the same day.

Before the next group

Check your backup numbers and staffing. For video groups, check location, callback number, privacy, and what happens if someone disconnects. Try your staff contact route before you need it.

Staff handoff

For staff only. Complete it in your program's clinical system if possible. A client's worksheet or any other client-held page is never the handoff.

From (name, role): _____ To (name, role): _____ Date and time: _____

What I saw and heard (exact words in quotation marks)

_____

_____

_____

_____

What I did, and how the person responded

_____

_____

_____

What I'm worried about, and what I don't know yet

_____

_____

_____

Next step, who does it, by when, and what happens if it doesn't happen

_____

_____

_____

Receiver: I understand and I'm taking on

_____

_____

Write only what was observed or reported. Don't copy forward claims of engagement, improvement, or consent that weren't there. Leave out other members' stories and names unless something specific is needed for safety.

Setting up your program

Fill this in once with your clinical lead, keep a copy in every facilitator's binder, and check it whenever staff or phone numbers change. Use roles and phone numbers, not client names.

Situation	Who I call, and how	Backup if no answer
Help during group (any concern)		
Possible suicide or self-harm risk		
Medical emergency, overdose, seizure, possible withdrawal		
Threat, violence, or an unsafe room		
Safeguarding or mandated-reporting question		
Medication question or possible side effect		
Interpreter or language support		
Video group: member disconnects or may be unsafe		

Before each group

□ I know who comes to stay with the group if I have to step out. If nobody can, I won't step out, and my plan for that is: _____

□ I have a made-up example ready for today's skill.

□ For video: I have each member's current location and a callback number, and I know the emergency service for that location.

□ I know where to write notes and where completed worksheets go.

Group size and staffing

Your program decides its maximum group size and when two facilitators are needed. Write it here so it isn't decided in the moment: _____

If your planned backup is missing, change the plan before group starts: find another person, shorten the session, or reschedule. An emergency that happens anyway still gets the emergency response.

Reviewed by: _____ Date: _____ Next review: _____